

# PUBLIC SUBMISSION

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**Comment On:** NSF\_FRDOC\_0001-3479  
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## General Comment

See attached file(s)

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## Attachments

Doug Hohulin RFI Office of Science and Technology Policy (OSTP) on the Development of an Artificial Intelligence (AI) Action Plan

March 15, 2025

Response to RFI pm the “Development of an AI Action Plan” [Regulations.gov](https://www.regulations.gov)

Dear OSTP Team,

I am Doug Hohulin President/Founder of Exponential Blueprint Consulting LLC. I work on AI projects “When the AI System Has to Be Right” AI in Healthcare, Regulation, Governance, Policy, Road Safety/AV, Education and Energy, Co-Author of [2030: A Blueprint for Humanity's Exponential Leap](#) [2030Book.org Website](https://2030Book.org) and [Tech Powered Healing: The Future of Medicine in the AI Age](#) and consultant for various companies including [AI & Partners](#) that provides an AI governance platform and AI literacy training that enables responsible use of AI. I am working on how to use AI to make America and the World Healthy - Saving 1 Billion Lives - Reducing Disabilities - by Empowering Patients & Clinicians With AI.

I support the priority policy actions needed to sustain and enhance America's AI dominance, and to ensure that unnecessarily burdensome requirements do not hamper private sector AI innovation. AI is a powerful tool that if used properly can be used to unlock trillions of dollars of economic value and productivity to the US and the world. But this powerful tool can be costly if not used properly. **In 2012, the Knight Capital Group suffered a \$440 million loss within ~30 minutes due to a malfunctioning algorithm.** One of the largest HIPAA settlements was \$16 million paid by Anthem, Inc. in 2018, following a cyberattack that exposed the electronic protected health information of ~79 million individuals. The use of AI could open the door for future cyberattack and exposing sensitive/proprietary information that could expose companies to even greater fines or liability.

As humanity uses AI to be our agents, companies must ensure they understand the opportunities, risks and benefits. Companies and individuals must know their liability (under current law and potential future laws) as they deploy these AI agents and tools. NTIA released the [AI Accountability Policy Report](#) offering policy recommendations to help support safe, secure, and trustworthy AI innovation. ISO/IEC 42001:2023 and the EU AI Act provide risk assessment guidelines that can be used to minimize risk and liability to a company – especially if they operate in areas outside the United States.

In December 2024, the [Bipartisan House Task Force releases report on AI innovation](#) ([AI-Task-Force-Report-FINAL.pdf](#)), the report highlighted different problems Americans and Industry faces and discussed ways AI and AI Policy could be used to solve the problems. Two problems highlighted.

- Treatment Errors (Rx) 2 Million Affected >250,000 Harmed Costs ~ \$20 Billion/Year
- Diagnostic Errors (Dx) Most Common Over 12 Million Affected, Most Catastrophic Estimated 4 Million Harmed; Most Costly Estimated > \$100 Billion/Year

[Paul Barach, B.Med.Sci, MD, MPH, Maj \(Ret.\), AUA](#) gave a recent presentation that highlighted the following problems in healthcare.

- “1 in 10 patients have poor quality and harmed in hospital care/ between 5.7 and 8.4 million deaths occurring annually from poor quality care ([Up to 8 Million Deaths Occur in Low- and Middle-Income Countries Yearly Due to Poor-Quality Health Care, Says New Report | National Academies](#))
- 20-40% health spending wasted due to poor quality of care and safety failures
- 15% of hospital costs being due to patient harms caused by adverse events”

As I wrote in the book [2030: A Blueprint for Humanity's Exponential Leap](#), “Dr. Jordan Shlain stated, “Healthcare at its best is a conversation between a doctor and a patient trying to solve a problem.” To be useful, AI must be harnessed to enhance the patient-doctor relationship (not replace it), improve preventative care, and address the modifiable risk factors that lead to chronic disease and early mortality. What if we can have an AI Health Coach to motivate us daily to work on helping us follow our clinician’s specified treatment plans? Fifty percent of people do not follow their treatment plans.”

“The National health expenditures (NHE) are unsustainable. New solutions are required to solve the challenges of Avoiding Avoidable Deaths and the cost of Healthcare. The [National Health Expenditure Projections, 2023–32: Payer Trends Diverge As Pandemic-Related Policies Fade | Health Affairs](#) By Jacqueline A. Fiore,

*“Health care spending growth is expected to outpace that of the gross domestic product (GDP) during the coming decade, resulting in a health share of GDP that reaches 19.7 percent by 2032 (up from 17.3 percent in 2022). National health expenditures are projected to have grown 7.5 percent in 2023 when the COVID-19 public health emergency ended.”* The report provides a summary of healthcare spending.

*Exhibit 1 - National health expenditures (NHE) and personal health care (PHC) expenditures, aggregate and per capita amounts, share of gross domestic product (GDP), and annual growth, selected calendar years 2019–3”*

| Category             | 2021       | 2024       | 2026       | 2032       |
|----------------------|------------|------------|------------|------------|
| NHE, billions        | \$4,289.1  | \$5,048.8  | \$5,560.3  | \$7,705.0  |
| PHC, billions        | \$3,561.5  | \$4,251.2  | \$4,687.4  | \$6,532.3  |
| GDP, billions        | \$23,594.0 | \$28,489.0 | \$30,798.9 | \$39,158.1 |
| NHE as a %of GDP     | 18.2%      | 17.7%      | 18.1%      | 19.7%      |
| Population, millions | 329.6      | 334.9      | 339.3      | 351.4      |
| NHE per capita       | \$13,012   | \$15,074   | \$16,387   | \$21,927   |

| <i>Category</i>       | <i>2021</i>     | <i>2024</i>     | <i>2026</i>     | <i>2032</i>      |
|-----------------------|-----------------|-----------------|-----------------|------------------|
| <i>PHC per capita</i> | <i>\$10,805</i> | <i>\$12,692</i> | <i>\$13,815</i> | <i>\$18,590</i>  |
| <i>GDP per capita</i> | <i>\$71,579</i> | <i>\$85,055</i> | <i>\$90,771</i> | <i>\$111,436</i> |

**Any AI Action Plan should be evaluated on how well “The Plan” helps the healthcare industry solves the Treatment Errors (Rx) and Diagnostic Errors (Dx), encourages the use of AI to enhance the patient-doctor relationship (not replace it) and lower the cost of healthcare while providing high quality care.**

The MIT A Risk Repository is an [AI Incident Tracker](#) – the “visualization shows how incidents of harm from AI reported in the [AI Incident Database](#) are increasing over time, with the greatest increase in incidents associated with the Misinformation and Malicious Actors domains from the MIT AI Risk Repository.” “The Plan” should be measured by how well it reduces harm and incidents like these while helping innovation.

As highlighted by the following surveys, AI is still limited in how it is being deployed in the workplace and by the Clinicians.

- [U.S. Workers Are More Worried Than Hopeful About Future AI Use in the Workplace | Pew Research Center](#)
- [AI Is in the Doctor’s Bag—And Primary Care Is Ready to Use It | Rock Health](#)

“You see the computer revolution everywhere except the productivity statistics.” - Robert Solow 1987. The question is when will the US see innovation and productivity gains from AI and how can The Plan maximize these gains?

I support the [Establishing the President's Make America Healthy Again Commission – The White House](#) but this order did not reference using AI to help support this order. AI could be a powerful tool to help “Make America Healthy Again.”

To enhance America's AI dominance, and to ensure that unnecessarily burdensome requirements do not hamper private sector AI innovation. The following areas should be considered:

- Encourage specific industry AI self-regulation.
- Encourage industry self-risk assessment to help companies innovate to solve problems but also deploy safe and effective AI. Encourage companies to leverage NTIA, ISO/IEC 42001:2023 and the EU AI Act risk assessment guidelines and best practices.
- US Government policy, regulations and laws should support innovation while helping companies minimize risk. Any new policies, regulations and any new laws must balance innovation, risk and liabilities.

- Encourage companies to understand their liabilities when they use AI tools to minimize their risk. Encourage industry to keep an expert human in the loop to monitor and control the AI agent to make sure it does not cause harm.”
- Encourage AI Literacy to companies and the American public so that all Americans can benefit from AI Innovation.
- Leverage AI to help “Make America Healthy Again.”

When I worked for Nokia, I created the North American Automated Vehicle/Connected Vehicle (AV/CV) strategy plan and for 4 years, I engaged in the AV/CV industry partners development as part of the 5GAA (<http://5gaa.org/>) Business Models and Go-To-Market Strategies Working Group, FCC Task Force and the US Federal, State and Local Workstream leadership team. In 2016, I was on the editing teams for the 5GAA and NGMN response to the US DOT/NHTSA V2V NPRM. See the response [5GAA comments to the NHTSA on notice of proposed rulemaking - 5GAA](#). I was part of the team that reviewed the ~80 company / agency / key organization responses. There were also ~400 individual responses from the public.

I support NSF and OSTP for the development of this AI Plan that “ensures an American Golden Age of technological innovation and economic prosperity.” I welcome the opportunity to engage with the administration to promote a responsible approach to AI that protects consumers and industry (by focusing on minimizing risk and liability) without stifling innovation and leverage AI to help “Make America Healthy Again.”

Respectfully,

Doug Hohulin, President/Founder of Exponential Blueprint Consulting LLC



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